
Intelligence Report

A Study of Eltrekibart and Mirikizumab in Adult Patients With Moderately to Severely Active Ulcerative Colitis

LEAD SPONSOR**Eli Lilly and Company****NCT ID****NCT06598943****DATE GENERATED****2026-03-13**

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Summary

Generated: 2026-03-13

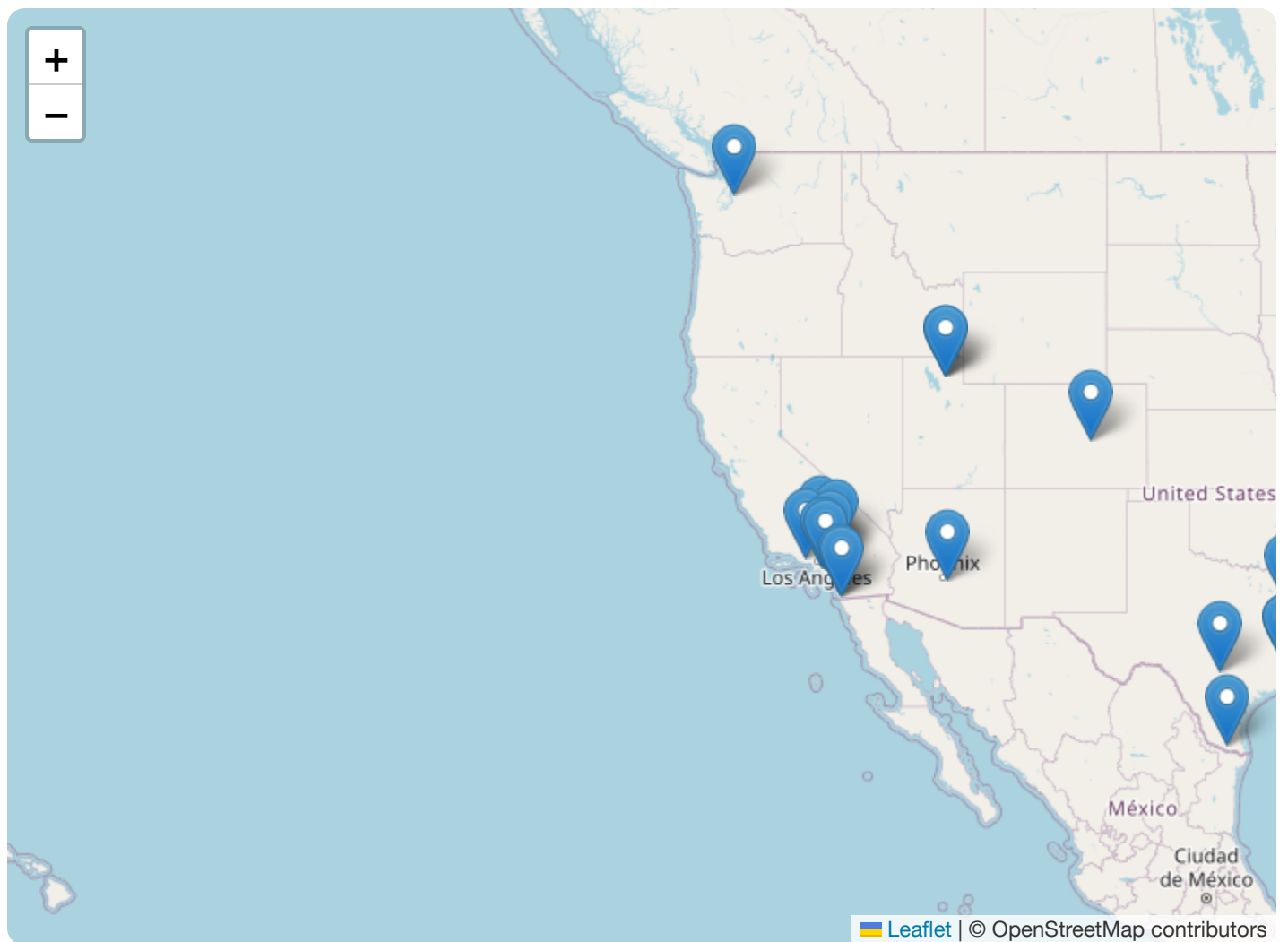
Summary	Details
Lead sponsor	Eli Lilly and Company
Condition	Ulcerative Colitis
Study type	INTERVENTIONAL
Phase	PHASE2
Enrollment	140.0
Inclusion points	5.0
Exclusion points	5.0
Locations	207.0
Patients per site per month	< 0.1

- With 207 locations and an enrollment of 140, your trial exhibits a highly decentralized model, resulting in a low recruitment density of 0.0145 patients per site per month.
- The trial's 46.8-month duration is significantly longer than the 12-month duration of direct competitors like Jina Pharmaceuticals (NCT06867042), suggesting a more extended clinical development timeline.
- While your trial maintains a balanced inclusion/exclusion profile (5 points each), other trials in the landscape show high variability, ranging from as few as 2 inclusion points to as many as 43 exclusion points, indicating diverse stringency in patient selection across the field.
- Compared to single-site trials that achieve up to 5.29 patients per site per month, your multi-site strategy will require rigorous site management to improve the current yield of 0.6763 patients per site.

 ClinicalTrials.gov extracted study record.

Diagnostics: Stored report loaded.

Map



- ✓ ClinicalTrials.gov extracted site records.
- ✓ OpenStreetMap basemap.

Similar trials

☐ Include only trials with at least 1 US site

Showing Phase PHASE2 interventional trials for ulcerative colitis, studying a drug, Recruiting or Not yet recruiting.

NCT ID	Lead sponsor	Enrollment	Sites	Duration	Patients per Site per Month	Map
NCT06213857	Tanta University	44.0	2.0	4.0	5.5	View
NCT06606808	University Medical Center Groningen	18.0	1.0	12.8	1.4	View
NCT06421818	The Second Hospital of Nanjing Medical University	144.0	1.0	32.0	4.5	View
NCT05466890	Palatin Technologies, Inc	16.0	14.0	30.5	< 0.1	View
NCT06764615	Takeda	183.0	16.0	55.1	0.2	View
NCT05177835	Abivax S.A.	203.0	46.0	52.0	< 0.1	View
NCT05731128	Sanofi	68.0	77.0	49.3	< 0.1	View
NCT06254950	Takeda	207.0	140.0	40.2	< 0.1	View
NCT05907330	Curacle Co., Ltd.	45.0	0.0	22.0	N/A	View
NCT05287126	Pfizer	36.0	47.0	103.7	< 0.1	View
NCT06619990	Xencor, Inc.	270.0	40.0	50.8	0.1	View
NCT05327790	University of Alberta	40.0	1.0	45.9	0.9	View
NCT07297069	Asian Institute of Gastroenterology, India	60.0	1.0	24.0	2.5	View
NCT06257875	AbbVie	156.0	191.0	41.3	< 0.1	View
NCT06867042	Jina Pharmaceuticals Inc.	150.0	0.0	12.0	N/A	View
NCT06850727	Odyssey Therapeutics	57.0	32.0	17.0	0.1	View
NCT06667934	The Second Affiliated Hospital of Anhui University of Traditional Chinese Medicine	60.0	0.0	6.1	N/A	View
NCT03869905	Muhammad N Aslam, MD	40.0	1.0	99.8	0.4	View
NCT07295834	Imperial College London	70.0	4.0	24.0	0.7	View
NCT07427017	Northwestern University	180.0	1.0	34.0	5.3	View
NCT07012395	Spyre Therapeutics, Inc.	645.0	126.0	33.2	0.2	View
NCT06636656	Boehringer Ingelheim	45.0	41.0	38.7	< 0.1	View
NCT03804931	Guangzhou First People's Hospital	120.0	1.0	143.4	0.8	View
NCT06420375	Brigham and Women's Hospital	20.0	1.0	26.2	0.8	View
NCT07186101	Eli Lilly and Company	252.0	147.0	39.7	< 0.1	View
NCT04925973	McMaster University	24.0	1.0	30.0	0.8	View
NCT06353828	CannaMore Biotech	45.0	1.0	25.1	1.8	View
NCT06979336	Genentech, Inc.	224.0	51.0	37.1	0.1	View
NCT07335055	Haisco Pharmaceutical Group Co., Ltd.	150.0	1.0	36.0	4.2	View
NCT04373473	The University of Texas Health Science Center, Houston	58.0	1.0	87.6	0.7	View
NCT06117423	University Medical Center Groningen	21.0	1.0	15.1	1.4	View

NCT ID	Lead sponsor	Enrollment	Sites	Duration	Patients per Site per Month	Map
NCT07064707	Alexandria University	60.0	1.0	15.3	3.9	View
NCT07300553	Assistance Publique - Hôpitaux de Paris	45.0	1.0	19.0	2.4	View
NCT07222189	Sanofi	325.0	0.0	48.0	N/A	View
NCT06127043	AnaptysBio, Inc.	132.0	100.0	28.9	< 0.1	View

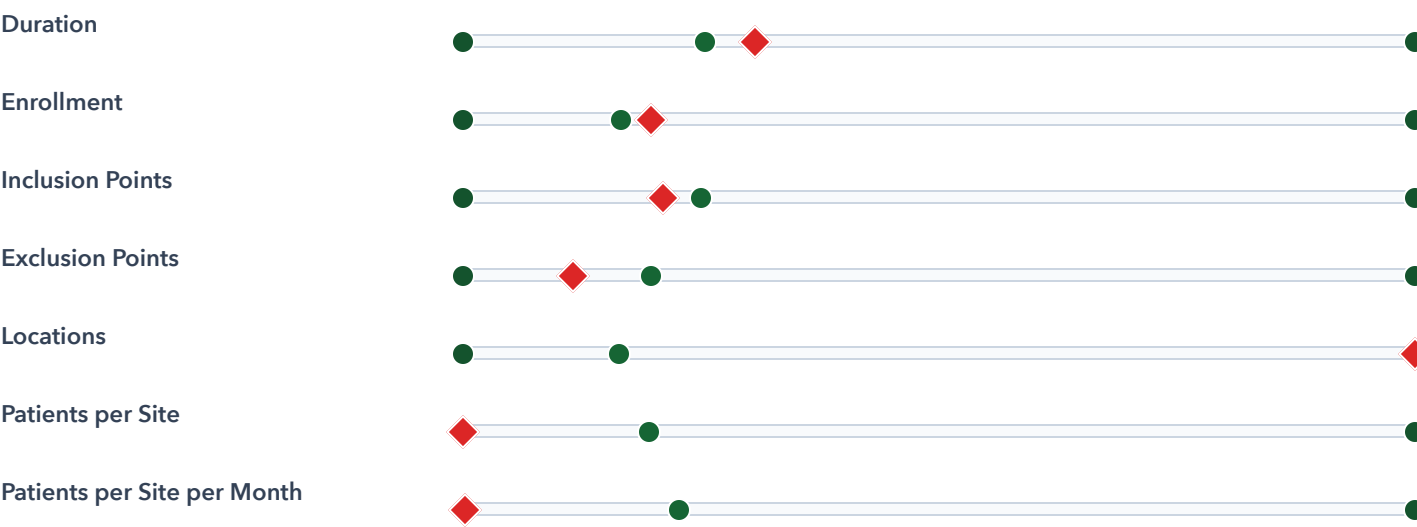
- ✓ ClinicalTrials.gov extracted study records.
- ✓ Zlick-related-trial matching and saved comparison dataset.

Key metrics comparison

☐ Include only trials with at least 1 US site

Similar trials found: 35 ?

Metric	My Trial	Min	Average	Max
Duration	46.8	4.0	39.4	143.4
Enrollment	140.0	16.0	120.4	645.0
Inclusion Points	5.0	1.0	5.7	20.0
Exclusion Points	5.0	0.0	8.5	43.0
Locations	207.0	0.0	31.1	191.0
Patients per Site	0.7	0.8	35.7	180.0
Patients per Site per Month	< 0.1	< 0.1	1.3	5.5



Legend: green circle = average, red diamond = My Trial, end circles = min/max.

- ✓ ClinicalTrials.gov extracted study records.
- ✓ Zlick-derived benchmark calculations from matching trials.

Closest competitors

Showing competing trials with sites in the same city than my trial.

NCT ID	Lead sponsor	Site overlap ?	Map
NCT06257875	AbbVie	30.9%	Link
NCT06127043	AnaptysBio, Inc.	27.1%	Link
NCT06254950	Takeda	23.7%	Link
NCT07186101	Eli Lilly and Company	18.8%	Link
NCT07012395	Spyre Therapeutics, Inc.	16.9%	Link
NCT05177835	Abivax S.A.	13.5%	Link
NCT06619990	Xencor, Inc.	12.6%	Link
NCT06636656	Boehringer Ingelheim	12.1%	Link
NCT05287126	Pfizer	10.1%	Link

US sites only

In-site competition

Showing which of my sites are also running competing trials.

Facility	City	State	Other Running Trials	Example NCT IDs
Peak Gastroenterology Associates	Colorado Springs	Colorado	3	NCT06636656, NCT06979336, NCT07186101
Tyler Research Institute	Tyler	Texas	3	NCT06254950, NCT06764615, NCT07186101
Baylor College of Medicine	Houston	Texas	2	NCT06257875, NCT06636656
Intergrity Research	Houston	Texas	2	NCT06257875, NCT06636656
Florida Research Institute	Lakewood Rch	Florida	2	NCT06636656, NCT07186101
University of Washington School Medicine	Seattle	Washington	2	NCT06254950, NCT06636656
Northwestern University	Chicago	Illinois	1	NCT07427017
Om Research, LLC	Lancaster	California	1	NCT07186101
Advanced Research Institute - South Ogden	Ogden	Utah	1	NCT07186101
Care Access-Ogden	Ogden	Utah	1	NCT07186101
GCP Clinical Research, LLC	Tampa	Florida	1	NCT06257875
International Center for Research LLC	Tampa	Florida	1	NCT06257875
Nodal Medical Center	Tampa	Florida	1	NCT06257875
Research Bay, Inc.	Tampa	Florida	1	NCT06257875

Facility	City	State	Other Running Trials	Example NCT IDs
Digestive Disease Medicine of Central New York	Utica	New York	1	NCT07186101

✓

 ClinicalTrials.gov extracted study records.

✓

 Zlick-derived city-level overlap and facility matching across active related trials.

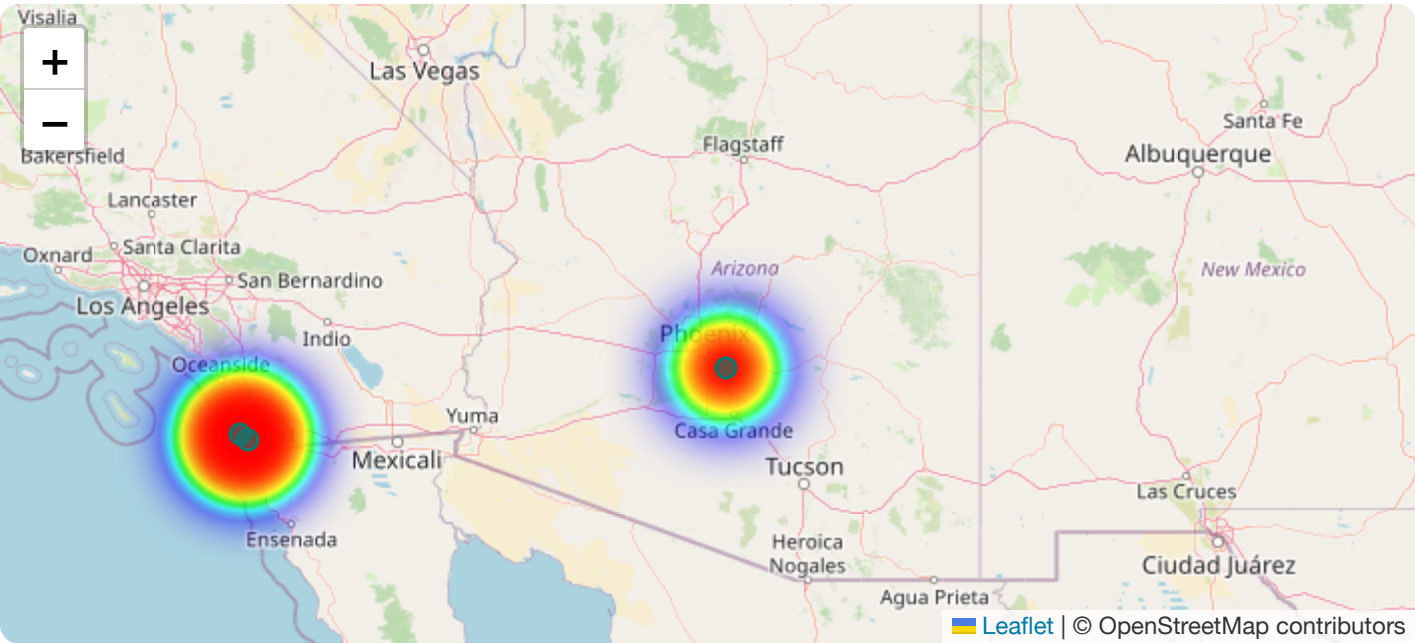
Ulcerative Colitis research nearby | Industry-sponsored

Showing PIs conducting relevant research near your sites. It also shows who they're working with and whether those sponsors have trials competing with yours.

Year: 2024 | Specialty lane: Gastroenterology

Closest site	Provider	Specialty	Distance	Total payments	Entities	Top payer	Competitor overlap
AZ Gastro Care - Chandler	JOSEPH DAVIS	Internal Medicine, Gastroenterology	On site	\$1,404,266	26	ModernaTX, Inc.	AbbVie, Boehringer Ingelheim, Pfizer
	SANJAY AHLUWALIA	Internal Medicine, Gastroenterology	On site	\$8,093	10	Eli Lilly and Company	AbbVie, Eli Lilly and Company, Pfizer
	LAYTH AL-JASHAAMI	Internal Medicine, Gastroenterology	17 mi	\$2,399	19	AbbVie Inc.	AbbVie, Pfizer, Takeda
	JAMES BURGESS	Internal Medicine, Gastroenterology	On site	\$1,994	23	AbbVie Inc.	AbbVie, Boehringer Ingelheim, Pfizer
	MATTHEW BALDUS	Internal Medicine, Gastroenterology	On site	\$1,700	18	AbbVie Inc.	AbbVie, Takeda
Preferred Research Partners	JOHN BABER	Internal Medicine, Gastroenterology	On site	\$173,636	17	Vanda Pharmaceuticals Inc.	AbbVie, Pfizer, Takeda
	ANGELO COPPOLA	Internal Medicine, Gastroenterology	On site	\$68,269	18	Genentech, Inc.	AbbVie, Genentech, Inc., Pfizer
	DAVID BACKSTEDT	Internal Medicine, Gastroenterology	On site	\$20,725	17	GastroGPO, LLC	AbbVie, Pfizer, Takeda
	TERENCE ANGTUACO	Internal Medicine, Gastroenterology	On site	\$1,639	14	AbbVie Inc.	AbbVie, Pfizer, Takeda
	MEER ALI	Internal Medicine, Gastroenterology	On site	\$1,280	3	Enterra Medical, Inc.	Takeda
Erick Alayo Medical Corp/Gastro SB Clinic	ERICK ALAYO	Internal Medicine, Gastroenterology	On site	\$306,858	10	Genentech, Inc.	AbbVie, Genentech, Inc., Takeda
	IVAN CUBAS	Internal Medicine, Gastroenterology	On site	\$350	8	AbbVie Inc.	AbbVie, Pfizer
	PATRICK SWEET	Internal Medicine, Gastroenterology	93 mi	\$156	2	Madrigal Pharmaceuticals	AbbVie
	JOHN DUQUE	Internal Medicine, Gastroenterology	On site	\$103	5	Evoke Pharma, Inc.	AbbVie
	GREGORY WIENER	Internal Medicine, Gastroenterology	On site	\$256,149	8	Vanda Pharmaceuticals Inc.	None

Closest site	Provider	Specialty	Distance	Total payments	Entities	Top payer	Competitor overlap
Southern California GI and Liver Centers (SCLC)	TAREK HASSANEIN	Internal Medicine, Gastroenterology	On site	\$652,087	21	Bausch Health US, LLC	AbbVie, Boehringer Ingelheim, Pfizer
	PREETI RESHAMWALA	Internal Medicine, Gastroenterology	On site	\$24	1	FUJIFILM Healthcare Americas Corporation	None



- ✓ CMS Open Payments 2024 payments grouped by covered recipient and reporting entities
- ✓ CMS Open Payments 2024 payments grouped by covered recipient and nature of payments
- ✓ Condition-relevant provider search from CMS NPI Registry

Specialty lane: Gastroenterology | Candidate NPIs scanned: 47

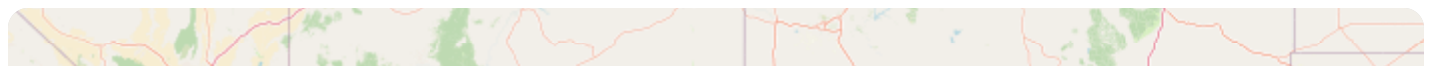
- ✓ CMS Open Payments 2024 payments grouped by covered recipient and reporting entities
- ✓ CMS Open Payments 2024 payments grouped by covered recipient and nature of payments
- ✓ Condition-relevant provider search from CMS NPI Registry

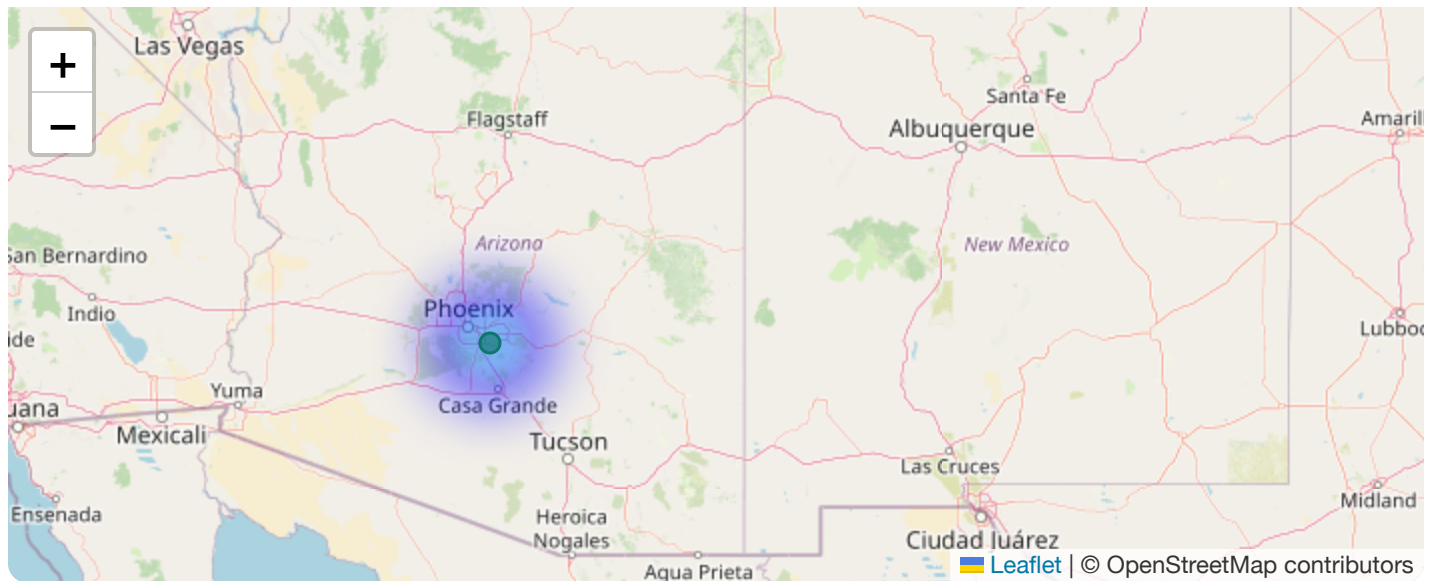
Ulcerative Colitis research nearby | NIH/public-funded

Showing research centers conducting relevant studies near your sites (funded by NIH).

Condition query: Ulcerative Colitis

Closest site	Institution	Lead PI	Recent grants	Total NIH funding	Latest FY	Example project
Erick Alayo Medical Corp/Gastro SB Clinic	CEDARS-SINAI MEDICAL CENTER	Gil Y Melmed	8	\$4,621,716	2025	Targeting the Mycobiome in Genetically Defined Patients with Crohns Disease
	AVERTO MEDICAL INC	Kenton Dodyan Fong	3	\$2,959,491	2025	Safety and Efficacy Trial of the ColoSeal ICD System for the Treatment of Rectal Cancer Patients
	STANFORD UNIVERSITY	CALVIN J KUO	7	\$2,413,633	2025	Impaired cellular RNA editing as a cause of inflammation in inflammatory bowel disease
	UNIVERSITY OF CALIFORNIA, SAN DIEGO	John T Chang	4	\$2,190,327	2025	Immune adaptations in acute pouchitis
	VIVREON BIOSCIENCES, LLC	Milton L Greenberg	2	\$1,999,994	2025	Predevelopment of VV8220, a Gut-selective CRAC Channel Therapeutic for Ulcerative Colitis
Southern California GI and Liver Centers (SCLC)	CEDARS-SINAI MEDICAL CENTER	Gil Y Melmed	8	\$4,621,716	2025	Targeting the Mycobiome in Genetically Defined Patients with Crohns Disease
	AVERTO MEDICAL INC	Kenton Dodyan Fong	3	\$2,959,491	2025	Safety and Efficacy Trial of the ColoSeal ICD System for the Treatment of Rectal Cancer Patients
	STANFORD UNIVERSITY	CALVIN J KUO	7	\$2,413,633	2025	Impaired cellular RNA editing as a cause of inflammation in inflammatory bowel disease
	UNIVERSITY OF CALIFORNIA, SAN DIEGO	John T Chang	4	\$2,190,327	2025	Immune adaptations in acute pouchitis
	VIVREON BIOSCIENCES, LLC	Milton L Greenberg	2	\$1,999,994	2025	Predevelopment of VV8220, a Gut-selective CRAC Channel Therapeutic for Ulcerative Colitis
Applied Research Center of Arkansas	UNIV OF ARKANSAS FOR MED SCIS	Teresa Jo Hudson	2	\$859,991	2023	Population-Based Analyses of Healthcare Utilization and Outcomes in Users of Medical Marijuana
Preferred Research Partners	UNIV OF ARKANSAS FOR MED SCIS	Teresa Jo Hudson	2	\$859,991	2023	Population-Based Analyses of Healthcare Utilization and Outcomes in Users of Medical Marijuana
AZ Gastro Care - Chandler	MAYO CLINIC ARIZONA	Elizabeth Carey	1	\$499,996	2022	A Prospective, Randomized, Multi-centered, Placebo-controlled Clinical Trial of Oral Vancomycin in Adults with Primary Sclerosing Cholangitis





- ✓ NIH RePORTER API v2 projects search
 - ✓ My Trial US site footprint from ClinicalTrials.gov extracted records
- Condition query: Ulcerative Colitis | Institutions matched: 13
- ✓ NIH RePORTER API v2 projects search
 - ✓ My Trial US site footprint from ClinicalTrials.gov extracted records

Competitive timeline



- ✓ ClinicalTrials.gov extracted study records.
- ✓ Zlick-derived timeline model from study start and completion dates.

Prevalence signal

County-level prevalence heatmap for Ulcerative Colitis, with optional overlay of My Trial US sites.

☐ Show My Trial US sites

Scoring prevalence signal, competitor density, referral support, CDC social vulnerability, HRSA shortage, and county-level Census access data.

Geography: state | Source: Secondary sources | Year: N/A

No state-level data for condition 'Ulcerative Colitis'. Checked other sources but no compatible direct location-level prevalence data was found. Attempts: CDC WONDER: No direct machine-query state prevalence endpoint configured for this automated workflow. | NHANES (NCHS): No state/county prevalence granularity for this automated map workflow.

- ✓ CDC PLACES prevalence data.
- ✓ ClinicalTrials.gov extracted My Trial US site records.

Protocol burden

Burden score: **91.3** (In line with similar trials). Score compares protocol complexity vs similar trials using duration and inclusion/exclusion criteria.

Factor	My Trial	Similar Trial Average	Difference
Study duration (months)	46.8	39.4	+18.7%
Inclusion criteria count	5.0	5.7	-12.9%
Exclusion criteria count	5.0	8.5	-40.9%

Scale: 100 = equal to similar-trial average burden; >100 = higher burden than peers; <100 = lower burden than peers.

-  ClinicalTrials.gov extracted study record.
-  Zlick-derived comparison against saved similar trials.

Predicted funnel

Eligibility-only US feasibility score (1 = very easy, 10 = very difficult).

Point-by-point criteria review

Quantifiable: 4 | Estimable: 4 | Not quantifiable: 2 | High bottleneck: 1

Type	Criterion	Category	US data signal / proxy	Why it may be a bottleneck	Bottleneck
Inclusion	Have moderately to severely active UC as assessed by the UC disease activity score.	Not quantifiable	Proxy using high-frequency medication changes or recent hospitalizations for UC flares.	UC disease activity scores (like Mayo or UCDAI) require clinical assessment and endoscopic findings not consistently captured in structured data.	High
Exclusion	Have a history of certain adenomas, dysplasia's, or malignancies.	Estimable	ICD-10 codes for neoplasm history.	Malignancy history is often available in structured data, but specific types of dysplasia may be buried in pathology reports.	Medium
Exclusion	Have had certain abdominal surgeries within the past 3 months or are likely to require surgery for UC during the study.	Estimable	CPT codes for abdominal surgery in the last 90 days.	Past surgeries are identifiable via CPT codes, but 'likelihood of future surgery' is clinical judgment.	Medium
Inclusion	Are on a stable dose of certain oral UC medications (including corticosteroids).	Estimable	Consistent dosage recorded in EHR over a 3-month period.	Stability can be inferred from prescription refill patterns and dosage consistency in EHR data.	Medium
Inclusion	Have an inadequate response to, loss of response to, or intolerance to at least one conventional medication (including corticosteroids) or one advanced therapy (including biologics, JAK inhibitors, or S1P immunomodulators).	Estimable	Pharmacy claims showing discontinuation of one therapy followed by initiation of another.	Requires identifying medication switches or additions, which can be inferred from pharmacy claims or EHR medication lists.	Medium

Type	Criterion	Category	US data signal / proxy	Why it may be a bottleneck	Bottleneck
Exclusion	Have a current diagnosis of Crohn's Disease or certain other inflammatory gastrointestinal diseases.	Quantifiable	Presence of ICD-10 K50.x codes.	Diagnosis codes for Crohn's (K50.x) are distinct from UC (K51.x).	Low
Exclusion	Have experienced a thrombotic event within 24 weeks before baseline.	Quantifiable	ICD-10 codes for DVT (I82.x) or PE (I26.x) within 6 months.	Thrombotic events (DVT/PE) are well-coded in medical claims.	Low
Exclusion	Have received anti-interleukin (IL)-23p19 or anti-IL-12p40 antibodies in the past.	Quantifiable	Claims for Ustekinumab, Risankizumab, Mirikizumab, etc.	Specific biologic therapies are identifiable via HCPCS/NDC codes in claims data.	Low
Inclusion	Have had an established diagnosis of UC of ≥3 months in duration before baseline.	Quantifiable	ICD-10 K51.x codes with date of first occurrence.	Diagnosis duration can be calculated from ICD-10 codes and initial diagnosis dates in EHR/claims data.	Low
Inclusion	Must meet contraception requirements.	Not quantifiable	None.	Contraception status is a behavioral/lifestyle factor not reliably documented in clinical databases.	Low

Predicted enrollment funnel is not available for this trial. Showing point-by-point criteria review instead.

Actual enrollment speed

Calculated using patients enrolled, number of sites, and start/end dates for similar trials reported as 'completed'.

Patients per site per month	Value
My Trial	< 0.1
Trials included	76.0
Mean	0.5
50th percentile	0.1
75th percentile	0.3
Minimum	0.0
Maximum	6.8

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The data reveals a significant disparity between the mean (0.4671) and the median (0.1022) patients-per-site-per-month, indicating a highly right-skewed distribution. With the maximum value of 6.8182 far exceeding the 75th percentile of 0.2832, the mean is heavily inflated by a small number of high-performing outlier sites. Consequently, the median provides a more accurate representation of typical site performance, as the majority of sites are enrolling patients at a rate significantly lower than the calculated average.

Given that 172 sites were excluded from this analysis, the current dataset likely represents only a subset of high-activity or specific-cohort locations. To improve future trial planning, stakeholders should prioritize the median performance metrics to establish realistic enrollment expectations rather than relying on the skewed mean. Further investigation into the operational characteristics of the top-performing outliers is recommended to determine if their recruitment strategies can be replicated across the broader, lower-performing site network.

In plain English: My Trial's estimate (0.01) is below the completed-trial average (0.47), so the target appears conservative versus historical enrollment speed.

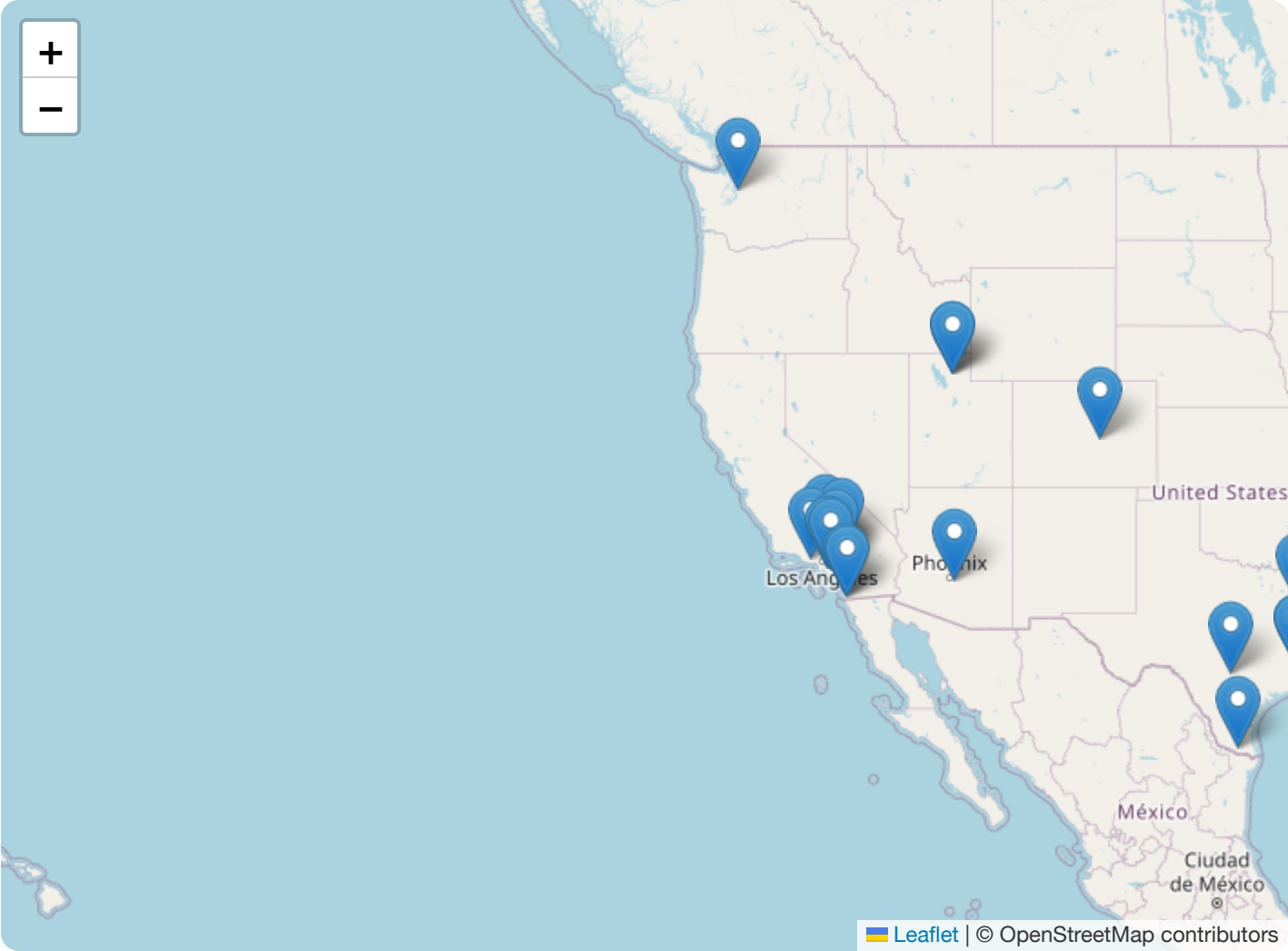
- ✓ ClinicalTrials.gov extracted study records for completed matching trials.
- ✓ Zlick-derived patients-per-site-per-month calculations.

Geographic hotspots

Showing areas with high prevalence of Ulcerative Colitis and low density of competing trials.

Map legend: blue = My Trial US sites, red circles = whitespace recommendations.

US sites: 46 | Geocoded My Trial sites: 45 | Whitespace areas: 0



This block loads automatically the first time you enter the Recruitment section.

- ✓ CDC PLACES (state) with county fallback, latest available year.
- ✓ ClinicalTrials.gov extracted study records for competitor density.

Find referring centers

Support enrollment in your sites with patient referrals from nearby specialists and treatment centers.

Selected site: Orlando Health Digestive Health Institute - Orlando, FL | ZIP: N/A

My site	Referrer	Type	Distance
Orlando Health Digestive Health Institute	DOCTORS OF CLINICAL SPECIALTIES LLC	Internal Medicine, Gastroenterology	4.2 mi
Orlando Health Digestive Health Institute	CFAGI, LLC	Internal Medicine, Gastroenterology	6.0 mi
Orlando Health Digestive Health Institute	STEVEN BRINT	Internal Medicine, Gastroenterology	10 mi
Orlando Health Digestive Health Institute	ABDUL BHUTTA	Internal Medicine, Gastroenterology	17 mi
Orlando Health Digestive Health Institute	ADVANCED MEDICAL GROUP OF CENTRAL FLORIDA, LLC	Internal Medicine, Gastroenterology	17 mi
Orlando Health Digestive Health Institute	MOHAMMAD ANWER	Internal Medicine, Gastroenterology	17 mi
Orlando Health Digestive Health Institute	SAEED ALI	Internal Medicine, Gastroenterology	24 mi
Orlando Health Digestive Health Institute	CURTIS ADOLPHSON	Internal Medicine, Gastroenterology	78 mi
Orlando Health Digestive Health Institute	PATRICK BRADY	Internal Medicine, Gastroenterology	78 mi
Orlando Health Digestive Health Institute	WOJCIECH BLONSKI	Internal Medicine, Gastroenterology	78 mi

Derived ZIP: N/A | Specialty lane: Gastroenterology | Specialty filter: Gastroenterology, Clinic/Center, Gastroenterology

- ✓ CMS NPI Registry.
- ✓ My Trial site location derived from ClinicalTrials.gov extracted site records.

Recommendations

Generate new

Action plan generated from the trial profile, benchmarks, competitive pressure, recruitment opportunity/risk, eligibility burden, and nearby research/referral evidence.

1. Consolidate the site footprint by closing or pausing the lowest-enrolling sites and concentrating resources on proven high-capacity locations.

High



The trial has 207 active locations versus a peer average of ~31, yet patients per site per month is <0.1 compared to a peer average of 1.3 – a 98.8% deficit – indicating extreme site dilution. Completed-actuals data (median 0.10 patients/site/month across 76 comparable trials) confirms that even well-run UC trials enroll slowly per site, so spreading across 207 sites without proportional enrollment is operationally inefficient. Reducing to a focused set of 40-60 top-performing sites would improve per-site economics and allow reallocation of monitoring and startup budgets toward patient-finding activities.

2. Leverage Southern California academic GI networks – particularly investigators connected to Cedars-Sinai and UC San Diego IBD programs – to accelerate referrals into nearby enrolled sites (Erick Alayo Medical Corp, Southern California GI and Liver Centers).

High



NIH research-network data shows Cedars-Sinai (PI Gil Y Melmed, 8 projects, \$4.6M funding) and UCSD (PI John T Chang, 4 projects, \$2.2M) as the highest-funded IBD research hubs proximate to two current trial sites in Southern California. These institutions have active UC-focused research pipelines (e.g., 'Impaired cellular RNA editing as a cause of inflammation in IBD') and established patient cohorts. Formalizing referral pathways from these academic centers to the nearby trial sites could tap a concentrated, trial-aware patient pool.

3. Mitigate patient competition at multi-trial sites (Peak Gastroenterology-Colorado Springs, Tyler Research Institute-Tyler TX, Baylor/Intergrity-Houston TX) by negotiating enrollment priority agreements or deploying dedicated recruitment support at those locations.

High



In-site competition data shows Peak Gastroenterology, Tyler Research Institute, and multiple Houston sites each running 2-3 competing UC trials (including Lilly's own NCT07186101 as well as AbbVie NCT06257875 and Takeda NCT06254950). With 35 related trials in the competitive landscape and the highest competitor overlap at 30.9% (AbbVie), these shared sites risk splitting an already narrow eligible population. Dedicated site-level coordinators or enhanced per-patient stipends at these contested sites can help secure referral priority for NCT06598943.

4. Differentiate the trial's value proposition to prospective participants by emphasizing the novel eltrekibart + mirikizumab combination mechanism and the relatively low protocol burden compared to competitors.

Medium



The protocol burden score of 91.3 ('in line with similar trials') with 40.9% fewer exclusion criteria than peers makes this trial comparatively accessible. Meanwhile, the eltrekibart + mirikizumab dual-mechanism design is unique in the UC landscape – web evidence from [clinicaltrials.eu](https://clinicaltrials.eu/trial/study-on-eltrekibart-and-mirikizumab-for-adults-with-moderate-to-severe-ulcerative-colitis/) and [allclinicaltrials.com](https://www.allclinicaltrials.com/study/NCT06598943) confirms this is the only active trial combining an anti-IL-13 (eltrekibart) with an anti-IL-23p19 (mirikizumab). Patient-facing recruitment materials should highlight the chance to receive two novel biologics and the streamlined eligibility, which may attract patients who have screened out of more restrictive competitor protocols.

5. Proactively plan for the endoscopy bottleneck by pre-scheduling central-read endoscopies and shortening the screening-to-randomization window at top sites.

Medium



The single high-bottleneck eligibility criterion – 'moderately to severely active UC as assessed by the UC disease activity score' – requires endoscopic confirmation that is not captured in structured data and adds scheduling complexity. Lilly's own parallel UC trial (NCT06937086 per [trial.medpath.com](https://trial.medpath.com/clinical-trial/aeee2d54bd1a297f/nct06936086-mirikizumab-tirzepatide-ulcerative-colitis-obesity)) uses the same modified Mayo/endoscopic subscore requirement, meaning endoscopy suites at shared sites face double demand. Pre-booking endoscopy slots and establishing rapid central-read turnaround agreements can reduce screen failures and the screening-to-enrollment lag that compounds the already low per-site enrollment rate.

Market analysis

Optional: Indicate a sponsor of focus

Enter company name here

Generate new

- The moderate-to-severe UC landscape is intensifying around the eltrekibart plus mirikizumab combination program.
- Janssen's guselkumab (Tremfya), a second anti-IL-23p19 antibody and direct mechanistic competitor to mirikizumab, launched for UC in late 2025 and is already establishing real-world share in the exact target population.
- The most credible near-term disruptor is Merck's tulisokibart (MK-7240), a first-in-class anti-TL1A antibody with FDA Breakthrough Therapy Designation and strong Phase 2 data, whose pivotal VIVID program readouts are expected mid-to-late 2026 with potential approval as early as H2 2027—introducing a novel mechanism that could reshape treatment sequencing before Lilly's combination data mature.
- Abivax's oral agent obefazimod remains on the radar but carries materially higher risk after its ABTECT-1 induction study missed its primary endpoint, making its regulatory path and timeline uncertain.
- Taken together, the combination of an entrenched new IL-23 competitor and a potentially fast-tracked TL1A entrant underscores the urgency of generating differentiated efficacy data for the eltrekibart-mirikizumab regimen to secure a compelling positioning narrative.

Recently launched	
Item	Commentary
Competitor	Guselkumab (Tremfya) – UC indication
Sponsor	Janssen / Johnson & Johnson
Related NCT(s)	NCT04033445, NCT04861350
Stage	Approved and launched (UC indication)
Regulatory / designations	['FDA approved September 2025 for moderate-to-severe UC', 'Previously approved for psoriasis and psoriatic arthritis']
Estimated market entry	US launch Q4 2025 - Guselkumab (Tremfya) received FDA approval for moderately to severely active ulcerative colitis in September 2025, based on the QUASAR Phase 3 program. As an anti-IL-23p19 antibody, it is a direct mechanistic competitor to mirikizumab and entered the UC market within the last 12 months. FDA approval in September 2025 based on QUASAR Phase 2b/3 data. PDUFA action date was met. Commercial availability followed approval.
Confidence	High FDA approval confirmed; product is commercially available for UC as of late 2025.

Recently launched	
Item	Commentary
Why relevant	Guselkumab is the second anti-IL-23p19 antibody approved for moderate-to-severe UC, directly competing with mirikizumab and any future eltrekibart combination. Targets the identical patient population. QUASAR Phase 3 program demonstrated efficacy in induction and maintenance. FDA approved September 2025. Subcutaneous formulation. Sources: s3, s4
Expected in 0-12 months	
Item	Commentary
Competitor	Vedolizumab IV Q4W (dose-intensification regimen)
Sponsor	Takeda
Related NCT(s)	NCT04738942
Stage	Phase 3 (Japan-specific)
Regulatory / designations	N/A
Estimated market entry	2026–2027 (Japan only, supplemental approval) - This is a Japan-specific Phase 3 trial evaluating vedolizumab IV every 4 weeks for patients with secondary loss of response on the standard Q8W regimen. Vedolizumab (Entyvio) is already a major marketed competitor globally. This dose-intensification label extension is narrow in scope (Japan only, secondary failures) but could retain patients in the vedolizumab franchise who might otherwise switch to newer agents. Phase 3 primary completion estimated mid-2025; Japan supplemental filing plausible 2026, approval 2026–2027.
Confidence	Low Narrow Japan-only scope with limited global competitive impact. Vedolizumab itself is already well-established. Incremental label change.
Why relevant	Vedolizumab is a major established UC competitor. Dose-intensification approval could retain patients who might otherwise switch to newer therapies like eltrekibart + mirikizumab. Phase 3 Japan-specific trial for Q4W dosing in secondary loss of response patients. Vedolizumab already approved globally for UC. Sources: s10, s11
Expected in 0-12 months	
Item	Commentary
Competitor	Tulisokibart (MK-7240)
Sponsor	Merck Sharp & Dohme LLC
Related NCT(s)	NCT06052059, NCT06052072
Stage	Phase 3 (pivotal)
Regulatory / designations	['Breakthrough Therapy Designation (FDA, for UC)']

Expected in 0-12 months	
Item	Commentary
Estimated market entry	H2 2027 - H1 2028 (regulatory submission likely late 2026 to mid-2027) - Tulisokibart is a first-in-class anti-TL1A monoclonal antibody in pivotal Phase 3 trials (VIVID-1, VIVID-2) for moderate-to-severe UC. Merck acquired the asset via its acquisition of Prometheus Biosciences. Phase 2 data were highly positive. Phase 3 primary completion is expected around mid-to-late 2026. With Breakthrough Therapy Designation, an accelerated review is possible. However, given the need for both induction and maintenance data, a regulatory submission before late 2026 is unlikely, placing potential approval in H2 2027 at the earliest. This falls at the boundary of the 12-month window. Phase 3 primary completion estimated mid-to-late 2026; Breakthrough Therapy Designation supports priority review. Regulatory filing likely late 2026 to early 2027; approval plausible H2 2027 to H1 2028.
Confidence	Medium Breakthrough Therapy Designation and strong Phase 2 data support accelerated development, but Phase 3 readouts have not yet occurred. Filing and approval timing remain uncertain.
Why relevant	First-in-class anti-TL1A antibody targeting the same moderate-to-severe UC population. Novel mechanism could be highly disruptive. Breakthrough Therapy Designation signals FDA engagement and potential for accelerated approval. Acquired via Prometheus Biosciences (\$10.8B acquisition). Phase 2 ARTEMIS-UC showed statistically significant clinical remission. VIVID-1 and VIVID-2 Phase 3 trials enrolling. Breakthrough Therapy Designation granted by FDA. Sources: s5, s6, s7
Expected in 0-12 months	
Item	Commentary
Competitor	Obefazimod (ABX464)
Sponsor	Abivax S.A.
Related NCT(s)	NCT05507216, NCT05507229
Stage	Phase 3
Regulatory / designations	N/A
Estimated market entry	H2 2027 - H2 2028 (uncertain; dependent on maintenance data and regulatory filing) - Obefazimod is a first-in-class oral RNA-modulating agent. The ABTECT-2 Phase 3 induction study completed in 2025, but Abivax reported mixed results from the ABTECT program, with the ABTECT-1 induction study missing its primary endpoint in early 2025. The maintenance study is ongoing. Given the mixed induction results, the regulatory path is uncertain. If maintenance data are supportive and a filing proceeds, approval could come in the 2028 timeframe, but confidence is reduced by the clinical setback. Phase 3 induction data mixed (ABTECT-1 missed primary endpoint early 2025). Maintenance data pending. Regulatory filing timing highly uncertain. Conservative estimate places potential approval no earlier than H2 2027.

Expected in 0-12 months

Item	Commentary
Confidence	Low ABTECT-1 induction study missed its primary endpoint, creating significant uncertainty about the regulatory path. Maintenance data and potential re-analysis could change the outlook, but risk of program failure is elevated.
Why relevant	If approved, obefazimod would be a novel oral option for moderate-to-severe UC, offering convenience advantages over injectable biologics. Targets the same refractory patient population. ABTECT-1 Phase 3 induction missed primary endpoint (reported early 2025). ABTECT-2 induction completed. Maintenance study ongoing. Abivax has stated it is evaluating the path forward. Sources: s8, s9

No watchlist programs identified for the 12-24 month window.

- ✓ [s9] Abivax Corporate Communications – ABTECT Program Updates (sponsor_press_release)
- ✓ [s6] Merck Press Release – Prometheus Biosciences Acquisition and Tulisokibart Development (sponsor_press_release)
- ✓ [s8] ClinicalTrials.gov – ABTECT-2 (NCT05507216) (clinical_trial_registry)
- ✓ [s4] ClinicalTrials.gov – QUASAR Phase 3 (NCT04033445) (clinical_trial_registry)
- ✓ [s10] ClinicalTrials.gov – Vedolizumab Q4W Japan (NCT04738942) (clinical_trial_registry)
- ✓ [s5] ClinicalTrials.gov – VIVID-1 (NCT06052059) (clinical_trial_registry)
- ✓ [s7] FDA Breakthrough Therapy Designation – Tulisokibart (regulatory_designation)
- ✓ [s2] FDA Drugs@FDA – Omvoh (regulatory_approval)
- ✓ [s1] FDA Omvoh (mirikizumab) Approval Letter (regulatory_approval)
- ✓ [s3] FDA Tremfya UC Supplemental Approval 2025 (regulatory_approval)
- ✓ [s11] Takeda Entyvio Product Information (sponsor_website)